



POLICY

Medication: Nurse Initiated Nicotine Replacement Therapy

Scope (Staff):	All NMHS Mental Health (MH) Nursing Staff
Scope (Area):	All NMHS MH Inpatient Services

1. Aim

North Metropolitan Health Service Mental Health (NMHS MH) nursing staff may initiate Nicotine Replacement Therapy (NRT) for inpatients over the age of 18 who self-present to staff requesting NRT, or who require NRT due to nicotine withdrawal symptoms.

2. Risk

Patients may be at risk of experiencing symptoms of nicotine withdrawal if unable to access NRT in a timely manner.

3. Principles

On admission all patients should have the PMR 6 NMHS MH Nicotine Assessment and Management on Admission Form completed.

If required during the course of admission, the nurse in charge of the shift or the delegated registered nurse may initiate patches and immediate release NRT (lozenge, inhalator or spray) if a Medical Officer is unavailable. Multiple doses may be administered whilst awaiting medical review.

This must be reviewed by a Medical Officer within 24 hours.

4. Electronic Cigarettes

Electronic cigarettes (e-cigarettes) and other personal vaporisers for delivery of nicotine or other substances are not permitted to be used in any area where smoking is restricted. ([Electronic cigarettes in Western Australia](#))

5. Indication for offer of NRT

- Patients who agree to use NRT to reduce nicotine withdrawal symptoms during their admission regardless of intent to stop smoking.
- Patients experiencing smoking withdrawal symptoms.

6. Contraindications

- Known hypersensitivity to nicotine, menthol or other excipients, including aspartame in lozenges (avoid in phenylketonuria).
- Non tobacco users.

7. Precautions

- Patients who weigh less than 45kg, may require a lower dose, e.g. 14mg/24hr patch.
- When considering use of nicotine spray, patients will be assessed for risk to self and others with regard to keeping the spray on their person. If this is regarded as unsafe the spray will be labelled and stored in the medication trolley or the patient will be offered another form of short acting NRT.
- Patients over the age of 64, or patients with dementia and other cognitive impairments or on soft diet will be assessed as to their ability to use lozenges appropriately.
- Patients with chronic throat conditions or unstable asthma will be prescribed nicotine spray or inhalators with caution.
- Nicotine patches should not be used in patients with adverse reactions to adhesive dressings or on areas of eczema or broken skin.
- Nursing staff will consult a medical officer before initiating NRT for patients who are pregnant or lactating; or who have had cardiovascular events.
- If the patient is already using NRT on admission, nursing staff may continue to provide it.

NRT must not be initiated by nursing staff for patients under the age of 18. A Medical Officer will prescribe NRT in these circumstances.

8. Administration instructions

The administering nurse will:

- Complete or ensure a PMR 6 form has been completed prior to administering any NRT.
- Assess the patient for contraindications and precautions.
- Select and offer the appropriate form of NRT up to the maximum daily dose. (refer to [Appendix 1: Nicotine Replacement Flowchart](#); [Appendix 2: Nicotine Replacement Therapy Options](#)).
- Document provision of NRT in black ink on the 'Once Only, Pre-Medication and Nurse/Midwife Initiated Medicines' section of the front of the patient's WA Hospital Medication Chart (PMR 60A) and sign the entry.
- Ensure the Medical Officer reviews the patient within 24 hours and prescribes ongoing NRT and/or another pharmacotherapy as appropriate.
- If a Medical Officer is not available to review the patient a phone order must be received before further NRT can be administered to patient.

Once the patient has been reviewed by the Medical Practitioner:

- the Medical Practitioner will prescribe fast acting NRT on the Nicotine Replacement Therapy (NRT) PRN Chart (PMR 56) and nicotine patches or another pharmacotherapy (if required) on the WA HMC.

- Nursing staff will document all doses of prescribed fast acting NRT administered on the NRT PRN Chart (PMR 56). Where patients are given NRT sprays to hold on their person, the NRT PRN Chart will be annotated with the date and time the spray was given to the patient.

9. Monitoring requirements

The following must be monitored and documented in the integrated patient progress notes:

- Any smoking withdrawal symptoms.
- Any adverse effects of NRT.

10. Compliance and Evaluation

Inpatient supply, usage and documentation of NRT to be performed annually by the NMHS MH Pharmacy department and reported to NMHS MH Drugs and Therapeutics Committee, MH Clinical Governance Committee and MH Management Committee.

Related internal policies, procedures and guidelines

[NMHS MH Medication Prescribing, Administration & Management for Inpatients](#)
[NMHS MH Smoking Care Guideline](#)

References

[WA Guidelines for supporting involuntary mental health inpatients](#)
[WA Smoke Free Policy and Guidelines](#)
[WA Mental Health Act 2014](#)
[Electronic cigarettes in Western Australia](#)

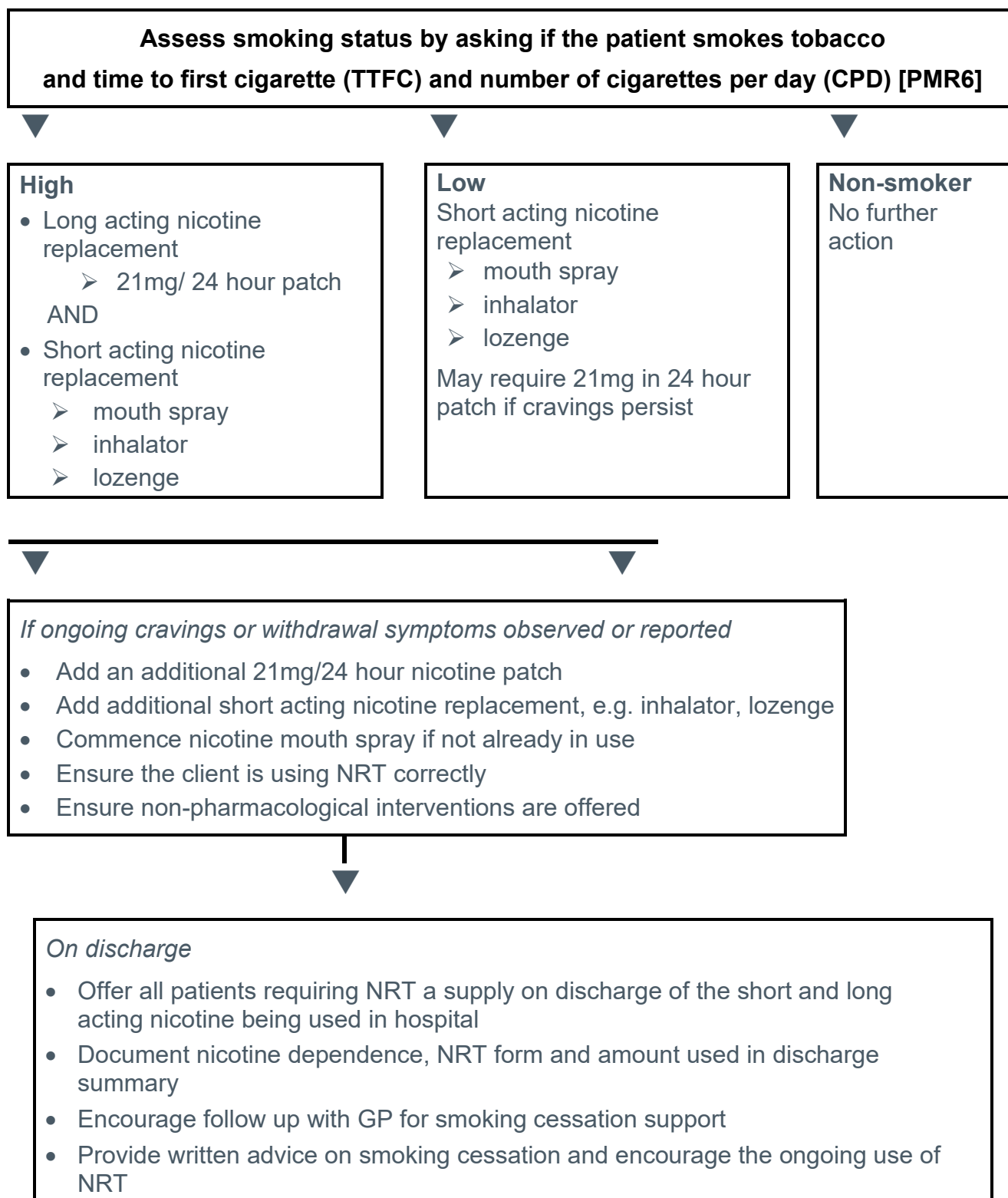
Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
Date First Issued:	23/04/2008	Last Reviewed:	14/07/2022	Review Date:	14/07/2025
Approved by:	NMHS MH Drugs and Therapeutics (DTC)			Date:	01/02/2022
Endorsed by:	Director of Clinical Services MHPHDS			Date:	14/07/2022
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 4: Medication Safety		☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 8: Governance, leadership & management		☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management				
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Document Version Control			
Date	Version	Author	Notes
20/07/2018	2.0	Chief Pharmacist, NMHS MH Chair, DTC	Scheduled review, new template, change of policy title, updated hyperlinks.
25/06/2019	2.1	Chief Pharmacist	Major Amendment: Review as part of Smoke Free Project, increase provision of doses from one dose to as many as required and types of NRT to include NMHS Mental Health Area Wide Policy Nurse Initiated Nicotine Replacement Therapy Page 4 of 6 patches, update hyperlinks and NSQHS standards, remove exclusion criteria and include indications, expand precautions to include form of NRT and medical conditions, expand nursing responsibilities to ensure revised documentation is completed and patient is reviewed as required. No change to scheduled review date (01/08/2021)
2022	3.0	Director of Nursing MHPHDS	New Template; links updated; updated information; addition of information regarding electronic devices.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD#1229.

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Appendix 1. Nicotine Replacement Flowchart



Appendix 2. NRT Options

NRT Options	Time to Peak Effect	Recommended Dose*	How to Use
Patches	2-6 hours	21mg/ 24 hours	Place a patch on clean, dry, hair free skin. Do not apply straight after a hot shower. Rotate the patch site daily to prevent build-up of adhesive. Nicotine patches should not be used in patients with adverse reactions to adhesive dressings or on areas of eczema or broken skin.
Mist spray	<5 minutes	1-2 sprays Maximum of 4 sprays/ hour	After priming, spray 1 or 2 pumps of the spray towards the inside of cheek or under tongue, avoiding the lips. The nicotine is absorbed through the lining of the mouth. To help absorption try not to swallow for a few seconds. Do not eat or drink when administering the spray. Use with caution in patients with chronic throat conditions or unstable asthma. Patients will be assessed for risk to self and others with regard to keeping the spray on their person.
Lozenge	20 minutes	2mg, 4mg Maximum one lozenge every 1-2 hours (2mg: low dependency 4mg: high dependency)	The lozenge takes about 20 minutes to dissolve; it needs to be moved from one side of the mouth to the other. As the lozenge dissolves in the mouth, nicotine is released into the saliva and absorbed through the buccal mucosa. The lozenge should not be sucked, chewed or swallowed whole. Patients should not eat or drink 15 minutes before use or while the lozenge is in the mouth. Patients over the age of 64, or patients with dementia and other cognitive impairments or on soft diet will be assessed as to their ability to use lozenges appropriately.
Inhalator	20-30 minutes	15mg cartridge Maximum one cartridge every 2 hours	Insert cartridge, close device to puncture. Each cartridge will deliver approximately 80 puffs before it is depleted. Inhale air through cartridge for 20 minutes, breathing in and out through the inhalator in short puffs. Patients should self-titrate dose according to withdrawal symptoms. Use with caution in patients with chronic throat conditions or unstable asthma. The nicotine in the cartridge will evaporate over 2 hours once removed from packaging.

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